

Colon & Colorectal Cancer — Clinical Trial & Treatment Timeline

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Clinical Summary

Recent developments in colorectal cancer research highlight several new clinical trials targeting advanced stages of the disease, particularly Stage IV and locally advanced rectal cancer. Notably, trials are investigating innovative treatment combinations, such as RC148 with chemotherapy and dual-target CAR-NK cells for biomarker-selected patients, which may provide new options for those with limited alternatives. Additionally, the exploration of low-dose radiotherapy combined with immunotherapy for locally advanced rectal cancer could enhance treatment effectiveness. Moving forward, it will be important to monitor the outcomes of these trials and their implications for treatment decisions in advanced colorectal cancer.

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Situation Summary

Events Detected: 10
 High Significance: 0
 Top Category: New Trial
 Primary Stage: Stage IV
 Primary Target: General
 Research Activity: Active Monitoring

Event Timeline

EVENT CLUSTER — Safety and Efficacy of Tegavivint in Patients With Metastatic Colorectal Carcinoma

MEDIUM **STAGE IV** **GENERAL** **NEW TRIAL** **VERIFIED** **PUBLISHED MAR 11**

Mar 11 19:09 · immediate · clinicaltrials.gov

A new trial is investigating the safety and effectiveness of Tegavivint for patients with advanced metastatic colorectal cancer, potentially offering a new treatment option for those with limited alternatives.

1 source(s) reporting · cluster 1 of 10

EVENT CLUSTER — A Phase II/III Clinical Trial of RC148 Plus Chemotherapy as 1L Therapy for Unresectable or Metastatic Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 19:09 · immediate · clinicaltrials.gov

A new clinical trial is testing the combination of RC148 and chemotherapy as a first-line treatment for patients with unresectable or metastatic colorectal cancer, potentially offering a new option for those facing advanced disease.

1 source(s) reporting · cluster 2 of 10

EVENT CLUSTER — Dual-Target CAR-NK Cells for Biomarker-Selected Advanced Colorectal Cancer

MEDIUM

STAGE IV

PD-1/PD-L1

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 19:09 · immediate · clinicaltrials.gov

A new clinical trial is investigating the use of dual-target CAR-NK cells in patients with advanced colorectal cancer, potentially offering a promising treatment option for those whose tumors express specific biomarkers.

1 source(s) reporting · cluster 3 of 10

EVENT CLUSTER — Low-Dose Radiotherapy to Sensitize Pucotenlimab Plus CAPEOX for pMMR Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 11 19:09 · immediate · clinicaltrials.gov

A new clinical trial is exploring the use of low-dose radiotherapy combined with the immunotherapy drug pucotenlimab and chemotherapy for patients with locally advanced rectal cancer, which may enhance treatment effectiveness and improve outcomes.

1 source(s) reporting · cluster 4 of 10

EVENT CLUSTER — A Clinical Study of Iparomlimab and Tuvonralimab Combined With Bevacizumab and Alternating Triweekly CAPOX/mCAPIRI Regimen as First-line Treatment for Unresectable Advanced Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 11 19:09 · immediate · clinicaltrials.gov

A new clinical trial is exploring a combination treatment of Iparomlimab, Tuvonralimab, and Bevacizumab with a chemotherapy regimen for patients with advanced colorectal cancer, potentially offering a new first-line option for those with unresectable disease.

1 source(s) reporting · cluster 5 of 10

EVENT CLUSTER — Everolimus in CDK12-Deficient Metastatic Colorectal Cancer (EVER-RECODE)

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED FEB 27

Mar 11 19:09 · immediate · [clinicaltrials.gov](#)

A new trial is exploring the use of Everolimus for patients with Stage IV colorectal cancer that has CDK12 deficiencies, potentially offering a new treatment option for this specific group.

1 source(s) reporting · cluster 6 of 10

EVENT CLUSTER — Effect of Perioperative Lidocaine or High-Dose Dexamethasone on Immune Response in Colon Cancer Surgery (PILDI Study)

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 6

Mar 11 19:09 · immediate · [clinicaltrials.gov](#)

The PILDI Study is investigating whether administering perioperative lidocaine or high-dose dexamethasone during colon cancer surgery can enhance the immune response, potentially improving outcomes for patients with Stage III colorectal cancer.

1 source(s) reporting · cluster 7 of 10

EVENT CLUSTER — FOLFOX Chemotherapy Combined With Fruquintinib and Serplulimab as First-Line Conversion Therapy for Initially Unresectable pMMR/MSS Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 11 19:09 · immediate · [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of FOLFOX chemotherapy with Fruquintinib and Serplulimab as a promising first-line treatment option for patients with initially unresectable stage IV colorectal cancer, potentially improving their chances for surgery and better outcomes.

1 source(s) reporting · cluster 8 of 10

EVENT CLUSTER — Neoadjuvant Chemoradiotherapy Plus Tislelizumab With or Without Probio-M9 in pMMR/MSS Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 11 19:09 · immediate · [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of neoadjuvant chemoradiotherapy with the immunotherapy drug Tislelizumab, potentially offering improved treatment options for patients with locally advanced rectal cancer.

1 source(s) reporting · cluster 9 of 10

EVENT CLUSTER — FOLFIRI and Bevacizumab With or Without Pelareorep for Second-Line Treatment of Metastatic RAS-Mutated, Microsatellite-Stable Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 11 19:09 · immediate · clinicaltrials.gov

A new clinical trial is exploring whether adding Pelareorep to the standard treatment of FOLFIRI and Bevacizumab can improve outcomes for patients with advanced colorectal cancer that has specific genetic characteristics.

1 source(s) reporting · cluster 10 of 10

Appendix A — Patient-Friendly Summary

Plain-language overview for patients, caregivers, and family members

What's happening

Recently, there have been some exciting developments in treatments for advanced colorectal cancer. A new trial is looking at a drug called Tegavivint to see how safe and effective it is for patients with metastatic colorectal cancer, which means the cancer has spread. Another trial is testing a combination of RC148 and chemotherapy as a first-line treatment for patients whose cancer cannot be surgically removed. There are also trials exploring new therapies using special immune cells and combinations of existing treatments, which could offer new hope for those facing advanced disease.

What this means for you

For patients, these developments may open up new options for treatment. If you or a loved one is dealing with advanced colorectal cancer, it might be worth asking your oncologist about these trials and whether they could be a good fit. It's important to discuss any new symptoms or changes in health with your doctor as well. Remember, research is always moving forward, and new treatments are being explored that could make a difference in your journey. Stay hopeful and keep the conversation going with your healthcare team.

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This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B — Colon & Colorectal Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year · ~52,000 deaths/year · 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s · Adults under 55 now account for ~20% of all new CRC diagnoses · Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG — Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years · FIT (fecal immunochemical test) annually · Cologuard every 1-3 years · CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative — whichever comes first · Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia → immediate evaluation regardless of age

Key Biomarkers — Test All mCRC at Diagnosis

RAS (KRAS/NRAS) · BRAF V600E · MSI/MMR status · HER2 · NTRK fusion · TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) → sotorasib + cetuximab · adagrasib + cetuximab

BRAF V600E (~8-10%) → encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) → pembrolizumab (1st line preferred) · nivolumab ± ipilimumab

HER2 amplified (~2-3%) → trastuzumab + pertuzumab · trastuzumab deruxtecan

RAS/BRAF WT → FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov · NCCN Guidelines (nccn.org) · Colorectal Cancer Alliance (ccalliance.org) · Fight Colorectal Cancer (fightcrc.org) · ACS (cancer.org)

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